

Considering the complaint filed on August 10, 2022, with this Agency by Ms. A.A.A. (hereinafter, the complainant), against MED&LASER; CLINICS, S.L. (hereinafter, the respondent), for not having duly attended to her right of access.

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: The complainant requested from the respondent entity:

- (1) the complete medical history identifying all the doctors and professionals who had intervened,
- (2) all complementary medical tests that had been performed,
- (3) all documents she has signed, including each session she has received, the informed consent, and any other document containing her signature, expressly indicating that her signature is a personal data, without her request having received the legally established response.

Attached are various contractual documents and invoices.

SECOND: In accordance with Article 65.4 of the LOPDGDD, which has provided a mechanism prior to the admission of complaints filed with the AEPD, consisting of forwarding them to the Data Protection Officers designated by the data controllers or processors, for the purposes provided for in Article 37 of the aforementioned regulation, or to them when they have not been designated, the complaint was forwarded to the respondent entity for analysis and to respond to the complainant and this Agency within one month, without having received a response from the respondent.

THIRD: The result of the transfer procedure indicated in the previous Fact did not allow for understanding the complainant's claims to be satisfied. Consequently, on November 10, 2022, for the purposes provided for in Article 64.2 of the LOPDGDD, the Director of the Spanish Agency for Data Protection agreed to admit the complaint for processing.

FOURTH: On December 12, 2022, the complaint was forwarded to the respondent for them to present any allegations they deemed appropriate, without any written submission having been received by this Agency.

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LEGAL GROUNDS

FIRST: The Director of the Spanish Agency for Data Protection is competent to resolve this matter, in accordance with the provisions of paragraph 2 of Article 56 in relation to paragraph 1 f) of Article 57, both of Regulation (EU) 2016/679 of the European Parliament and of the Council of April 27, 2016, on the protection of natural persons with regard to the processing of personal data and on the free movement of such data (hereinafter, GDPR); and in Article 47 of the LOPDGDD.

SECOND: In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as handling complaints filed by a data subject and investigating the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In the case that they have designated a data protection officer, Article 39 of the GDPR assigns to this officer

the function of cooperating with said authority.

In accordance with this regulation, prior to the admission of the complaint that gives rise to this procedure, the same was forwarded to the responsible entities for them to analyze it, respond to this Agency within one month, and prove that they have provided the complainant with the appropriate response, in the event of exercising the rights regulated in Articles 15 to 22 of the GDPR.

This admission agreement determines the opening of the present procedure for failure to attend to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, according to which:

“1. When the procedure refers exclusively to the failure to attend to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679, it will be initiated by an admission agreement, which will be adopted in accordance with the provisions of the following article.

In this case, the deadline for resolving the procedure will be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint to be upheld.”

It is not deemed appropriate to pursue administrative responsibilities within the framework of a sanctioning procedure, whose exceptional nature implies that, whenever possible, preference should be given to alternative mechanisms supported by current regulations.

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It is the exclusive competence of this Agency to assess whether there are administrative responsibilities that should be clarified in a sanctioning procedure and, consequently, the decision regarding its initiation, with no obligation to initiate a procedure upon any request made by a third party. Such a decision must be based on the existence of elements that justify the initiation of sanctioning activity, circumstances that do not occur in the present case, considering that with the present procedure the guarantees and rights of the claimant are duly restored.

THIRD: The rights of individuals regarding the protection of personal data are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, deletion, opposition, the right to limit processing, and the right to data portability are contemplated. The formal aspects related to the exercise of these rights are established in Article 12 of the GDPR and Article 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which will be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and to express its reasons in case it does not intend to comply with such request. The burden of proof for compliance with the obligation to respond to the request for the exercise of rights made by the affected party lies with the data controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language. Regarding the right of access to personal data, in accordance with the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected party to specify the "data or processing activities to which the request refers." The right will be considered granted if the data controller provides remote access to the data, with the request being deemed fulfilled (although the data subject may request information regarding the aspects provided for in Article 15 of the GDPR).

The exercise of this right may be considered repetitive more than once during a period of six months, unless there is a legitimate reason for it.

On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected party.

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FOURTH: In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, “the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data.”

Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the personal data have been or will be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others; that is, the right of access shall be granted in such a way that it does not affect third-party data.

The right of access in relation to medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which establishes the basic regulations governing Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal text of which states:

- “1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Healthcare centers shall regulate the procedure that guarantees the observance of these rights.
2. The patient's right of access to the medical record may also be exercised by duly accredited representation.
3. The patient's right of access to the documentation of the medical record may not be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.
4. Healthcare centers and individual practitioners shall only facilitate access to the medical records of deceased patients to individuals connected to them, for family or factual reasons, unless the deceased expressly prohibited it and this is evidenced. In any case, access by a third party to the medical record motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided.”

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

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- “1. The medical record shall incorporate information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a written record, or in the most appropriate technical support, of the information obtained in all their care processes,

carried out by the health service both in the primary care and specialized care settings.

2. The main purpose of the medical record is to facilitate healthcare, documenting all data that, in the medical opinion, allows for the truthful and updated knowledge of the health status.

The minimum content of the medical record shall be as follows:

- a) Documentation related to the clinical-statistical sheet.
- b) Admission authorization.
- c) Emergency report.
- d) Anamnesis and physical examination.
- e) Progress notes.
- f) Medical orders.
- g) Consultation sheet.
- h) Reports of complementary examinations.
- i) Informed consent.
- j) Anesthesia report.
- k) Operating room or delivery record report.
- l) Pathological anatomy report.
- m) Nursing care evolution and planning.
- n) Therapeutic nursing application.
- ñ) Vital signs chart.
- o) Clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) shall only be required in the completion of the medical record when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical record, in aspects related to direct patient care, shall be the responsibility of the professionals involved.

4. The medical record shall be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the healthcare professionals of the data of a specific patient in each care process.”

Regarding the preservation of the medical record, Article 17 of the LPA, in its points 1 and 5, states that:

“1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original support, for the proper care of the patient for the appropriate time for each case and, at a minimum, five years from the date of discharge from each care process...”

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5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate.”

FIFTH: In the case analyzed here, from the examination of the documentation provided, it is observed that the claiming party requested various matters from the responding entity, without having received a response.

(1) Regarding the complete medical record with the identification of all the doctors and professionals who have intervened, it is appropriate to uphold the right of access concerning the medical record. However, this does not include, in the terms provided in Article 15 of the GDPR and the provisions of Law 41/2002, the identification of the doctors or professionals who have intervened.

(2) In relation to the request for all complementary medical tests that have been performed, the claimant has the right to the complementary clinical tests carried out, as they are part of the medical record.

(3) Regarding all documents that have been signed, including each session received, the informed consent, and any other document containing the claimant's signature, it should be noted that the right

of access does not encompass the delivery of documents, but rather the facilitation of personal data that pertains to them.

Notwithstanding the above, in the present case, since it is not possible to provide only the signature, the claimant has the right to be provided with a copy of their personal data, handwritten signature, which is indissolubly linked to the documents they have signed.

Considering the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD the complaint filed by Ms. A.A.A. and to urge MED&LASER; CLINICS, S.L. with NIF B87482675 to, within ten working days following the notification of this resolution, send to the claiming party a certification addressing the requested right of access specified in the Fifth Legal Basis, or to deny it with justification indicating the reasons why the request cannot be met, in accordance with the provisions established in the body of this resolution. The actions taken as a result of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of the infringement considered in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to Ms. A.A.A. and to MED&LASER; CLINICS, S.L.

In accordance with the provisions established in Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

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Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution, or directly file a contentious-administrative appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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Mar España Martí

Director of the Spanish Agency for Data Protection

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